



PERINEAL INJURIES AND REPAIR

Maryam Bakir
Assistant Professor

Objective

- Understand the assessment and management of perineal tears.
- Understand the assessment and management of episiotomy.

INTRODUCTION

- The most common form of operative intervention is suturing of a perineal tear or episiotomy.
- 85% per cent of women who have a vaginal birth will have some degree of perineal trauma and 60–70% will require suturing.

INTRODUCTION

- Therefore the first important step following the birth of a baby and the delivery of the placenta is to assess the woman carefully to identify and classify any perineal tearing.
- A complete assessment includes both vaginal and rectal examination.

Classification of perineal tears

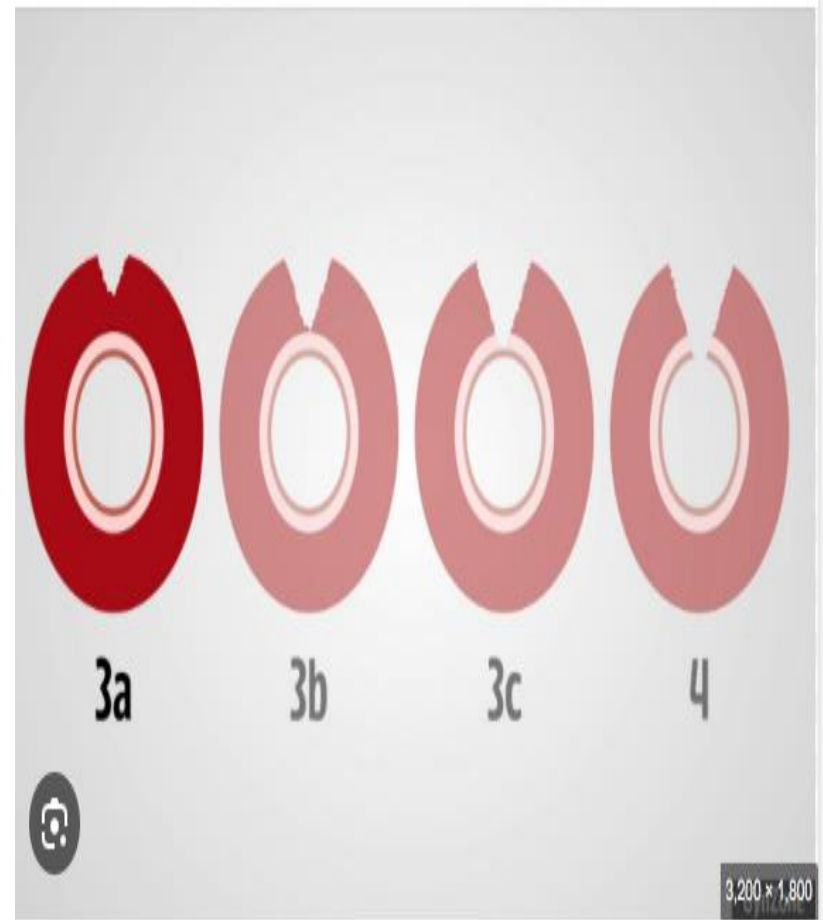
- **First-degree** tears are lacerations of the skin or vaginal epithelium only.
- **Second-degree** tears involve **perineal muscles** and includes episiotomy.

Classification of perineal tears



Classification of perineal tears

- **Third-degree** tears involve any part of the anal sphincter complex (**external or internal**):
 - 3a:** less than 50% of the **external** anal sphincter is torn
 - 3b:** more than 50% of the **external** anal sphincter is torn
 - 3c:** the tear involves the **internal anal sphincter** and/or complete disruption of the external sphincter
- **Fourth-degree** tears involve the anal sphincter complex extending into the **rectal mucosa**.



OASIs

- Third- and fourth-degree tears are grouped together and termed obstetric anal sphincter injuries (OASIs).
- OASIs are reported in approximately 3% of nulliparous and 0.5% of multiparous individuals.
- In general terms, **external** anal sphincter incompetence causes faecal **urgency**, whereas **internal** anal sphincter incompetence causes faecal **incontinence**.

Risk factors

Perineal tears occur more commonly with

- pro- longed labour, especially prolonged pushing,
- big babies,
- malposition such as occipito-posterior and
- in association with forceps or vacuum-assisted delivery.

Preventions

- **Perineal massage** can be performed in the ante- natal period and this may reduce the risk or extent of tearing.
- Similarly, the use of **warm compresses**, gentle perineal **support** and **controlled crowning** of the fetal head may reduce trauma at the time of birth.

Repair

- First-degree tears or minor lacerations with minimal or no bleeding may not require surgical repair and will heal spontaneously.
- Second-degree tears and episiotomies require surgical repair.

SURGICAL TECHNIQUE

- An **explanation** should be provided to the mother and verbal **consent** should be documented or, if a complex tear requires repair in an operating theatre, written consent should be gained.
- Adequate **analgesia** should be provided by infiltration with local anaesthetic or topping up an epidural.
- The operator should recheck the **extent** of lacerations with a vaginal and rectal examination.

SURGICAL TECHNIQUE

- Sometimes, the anatomy is distorted and assessment is improved following **positioning in lithotomy** with effective anaesthesia.
- If a tear is more complex than initially appreciated, a more **experienced operator** may be required.
- It may be helpful to place a pad or tampon high in the vagina to prevent blood loss from the uterus obscuring the view.

SURGICAL TECHNIQUE

- Care needs to be taken that this is removed at the end of the procedure.
- The vaginal **mucosa is repaired first** using rapidly absorbable suture material on a large, round body needle.
- A knot should be tied above the apex of the cut or tear and a continuous stitch should be used to close the vaginal mucosa.
- Interrupted sutures are then placed to close the **muscle** layer.

SURGICAL TECHNIQUE

- Closure of the **perineal skin** follows with either interrupted sutures or a continuous subcuticular stitch, which produces more comfortable results.
- A gentle **vaginal examination** should be performed to check for any missed tears and to ensure that good apposition has been achieved.

SURGICAL TECHNIQUE

- A **rectal examination** should be performed to confirm that the sphincter feels intact and to ensure that no sutures have been inadvertently placed through the rectal mucosa.
- If sutures are felt in the rectum they must be removed and replaced.
- The **pad or tampon should be removed** and a careful count of swabs, instruments and needles should be completed and documented in the records alongside the operation note and post-operative instructions. Analgesia should be prescribed.

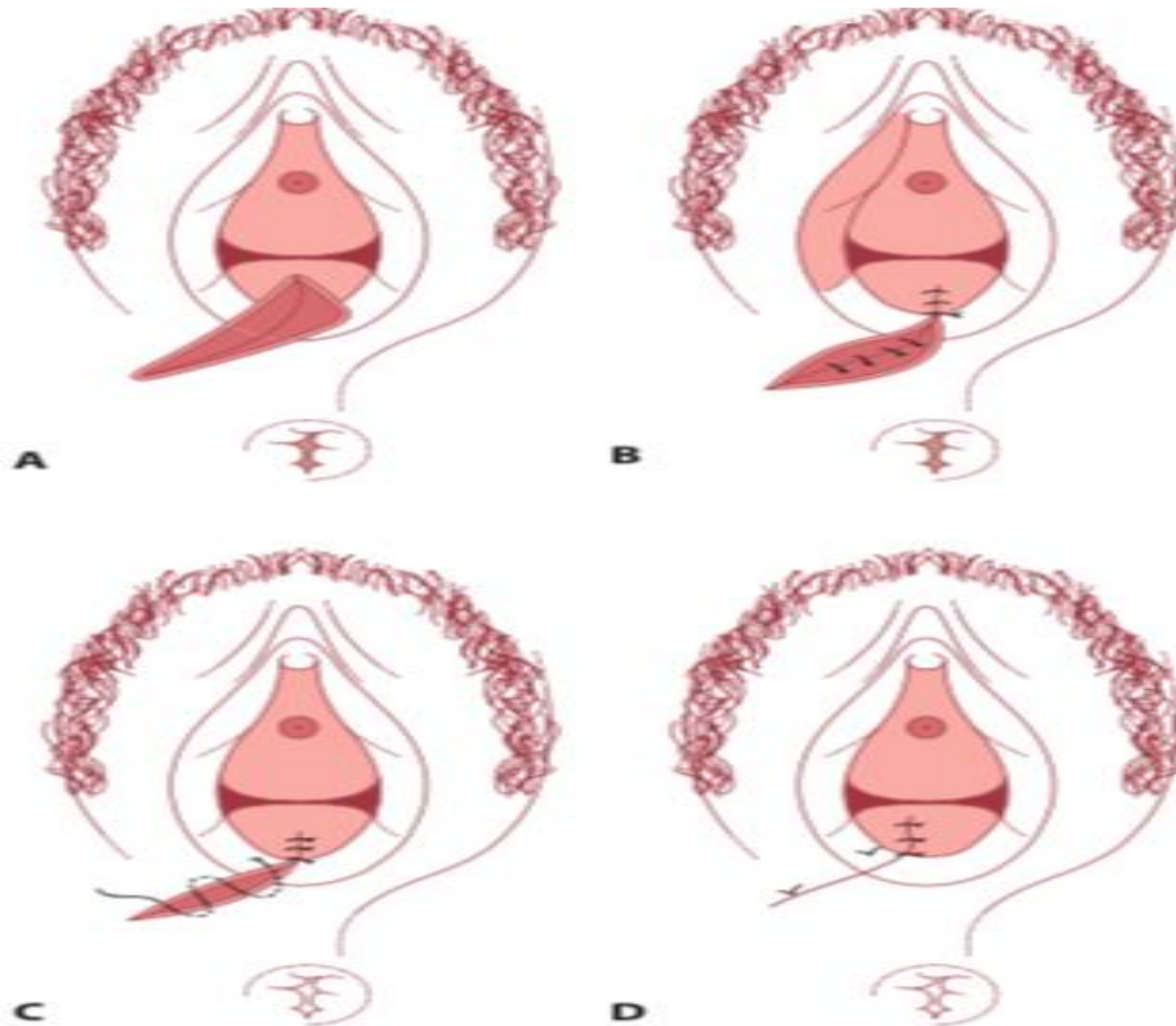


Figure 13.1 Repair of an episiotomy/second-degree perineal tear. (A) The perineum prior to the repair.

OBSTETRIC ANAL SPHINCTER INJURY REPAIR

- Repair of third- and fourth-degree tears should be performed or directly supervised by a trained practitioner.
- There must be adequate analgesia.
- In practice, this means either a regional or a general anaesthetic, as local infiltration does not allow relaxation of the sphincter enough to allow a satisfactory repair.
- The lighting must be adequate and an assistant is usually needed.
- Repair of the rectal mucosa and/or internal anal sphincter should be performed first.

OBSTETRIC ANAL SPHINCTER INJURY REPAIR

- The torn external sphincter is then repaired.
- It is important to ensure that the **muscle is correctly approximated** with **long-acting sutures** so that the muscle is given adequate time to heal.
- Some surgeons opt for **an end- to-end** repair, while others use an **overlap** technique; current evidence suggests that the outcome is similar with both methods.
- The remainder of the perineal repair is the same as for second-degree trauma.
- The surgical repair should be **documented** and a clinical incident form should be completed for risk assessment purposes.

OBSTETRIC ANAL SPHINCTER INJURY

AFTERCARE

- **Lactulose (laxative)** and a bulk agent, such as ispaghula husk, are recommended for **5–10 days** and the woman should remain in hospital until she has had a first bowel motion.
- An oral **broad-spectrum anti- biotic** should be prescribed for **5 to 7** days to reduce the risk of infection.
- Regular **oral analgesia** should also be prescribed.
- All women who have sustained a third- or fourth-degree tear should be **offered follow-up in the postnatal period**.
- At **6–12 weeks**, a full evaluation of the degree of symptoms should take place.

OBSTETRIC ANAL SPHINCTER INJURY AFTERCARE

- This must include careful **questioning** regarding **urinary and faecal** symptoms and advice in relation to future pregnancy and delivery.
- Asymptomatic women should be advised that the risk of recurrence in a future pregnancy **is 6–8%** and that vaginal birth is safely achievable.
- **Symptomatic** women should be offered investigation including **endoanal ultrasound** and **manometry**.
- Women with ongoing **troublesome symptoms** should be offered an **elective caesarean section** for future deliveries.

THANK YOU